

Office Visit — October 14, 2024

with Miriam Okafor-Stein, MD at Riverside Otolaryngology Associates, 88 Deverell Avenue, Suite 410, Brooklyn, NY 11205 · (718) 555-0164

Notes from Care Team

Progress Notes

Miriam Okafor-Stein, MD · Otolaryngology · Electronically signed 10/14/2024 4:32 PM

RIVERSIDE OTOLARYNGOLOGY ASSOCIATES
DIVISION OF RHINOLOGY & SINUS SURGERY

DOS: October 14, 2024
Name: Theo Marchbank
DOB: 1994-03-12
Address: Apt 3B, 412 Calver Street, Brooklyn, NY 11205
Phone: (212) 555-0147

Visit Type: New Patient Office Visit
Referring Provider: Dana Kellerman, MD

Reason for Visit:

- Nasal obstruction, left greater than right
- Recurrent sinus infections

"I get three or four sinus infections a year and my left nostril is pretty much always blocked, especially at night."

History of Present Illness:

30 y.o. male, new patient, referred by PCP for evaluation of recurrent rhinosinusitis. Reports 3-4 episodes per year x several years with facial pressure, purulent drainage, and congestion, most responding to antibiotics. Baseline left-sided nasal obstruction, worse supine, with mouth breathing and snoring reported by partner.

Denies epistaxis, anosmia, facial trauma as an adult. Recalls a soccer injury to the nose in his teens, not treated. OTC oxymetazoline used intermittently, no more than 3 days at a time. No prior sinus imaging. No prior nasal surgery.

Symptom — Nasal congestion: Yes
Symptom — Facial pain or pressure: Yes
Symptom — Post-nasal drainage: Yes
Symptom — Decreased sense of smell: No
Symptom — Nosebleeds: No
Symptom — Snoring: Yes
Symptom — Ear pain or fullness: No
Symptom — Hearing loss: No
Symptom — Hoarseness: No
Symptom — Nighttime mouth breathing: Yes

ALLERGIES: No known drug allergies on file

MEDICATIONS:

- fluticasone propionate (generic) 50 mcg/actuation nasal spray — 2 sprays each nostril daily; Disp: 16 g; Rfl: 3 — STARTED THIS VISIT
- oxymetazoline 0.05% nasal spray — patient-directed OTC use, counseled to limit to 3 consecutive days

VITALS:

BP: 118/74

Pulse: 66

Height: 5' 11" (180.3 cm)

Weight: 172 lb (78 kg)

BMI: 24.0

PHYSICAL EXAMINATION:

General: Well-appearing, no acute distress. Voice normal.

Ears: EACs clear bilaterally; TMs intact with normal light reflex.

Nose/anterior rhinoscopy: Caudal septum deviated to the LEFT with near-contact of the left inferior turbinate; bilateral inferior turbinate hypertrophy, mucosa boggy without polyps. No purulence at middle meatus today.

Nasal endoscopy (diagnostic, in-office): Confirms leftward septal deviation with a posterior septal spur; middle meatus patent bilaterally; nasopharynx clear, no mass.

Oral cavity/oropharynx: Moist mucosa, no lesions; tonsils 1+, symmetric.

Neck: No lymphadenopathy, no thyromegaly.

Impression/Summary:

1. **Deviated nasal septum (leftward, with posterior spur)** — likely post-traumatic, symptomatic
2. **Recurrent acute rhinosinusitis** — 3-4 episodes/year by history
3. **Inferior turbinate hypertrophy, bilateral** — contributing to obstruction

Plan/Recommendations:

1. Start intranasal fluticasone daily, both nostrils, with technique counseling; continue for a minimum of 8 weeks before assessing benefit. Add daily large-volume saline irrigation.
2. Discontinue habitual decongestant spray use; discussed rebound congestion risk.
3. If symptoms persist on maximal medical therapy, obtain non-contrast sinus CT and discuss septoplasty with inferior turbinate reduction. Surgical anatomy and expectations reviewed in general terms today.
4. Patient education: sleep-position measures, humidification in winter months.

Follow up: Return to clinic in 3 months to reassess on medical therapy, sooner if a sinus infection develops.